ANALYSTLAB AFRICA

EXPERIENCE LAB INTERNSHIP PROGRAMME

HEALTHCONNECT CLINIC EXPERIENCE LAB

WEEK 5 PROJECT SUMMARY

Improving Patient Appointment Attendance and Healthcare Support Using Data and AI

Document Control

Project Title	Improving Patient Appointment Attendance and Healthcare Support Using Data and AI
Track	Project Management
Project Phase	Solution Development & Implementation
Reporting Period	Week 5
Status	Evidence-Informed Execution Baseline Established

1. WEEK 5 PLANNED FOCUS

Week 4 established the project's management foundation, including its scope, stakeholder structure, work breakdown, dependencies, risks and communication approach.

Week 5 shifted the focus from establishing the project structure to controlling how work moved through it.

The primary management objective was to translate approved work packages into actionable activities, establish ownership and priorities, monitor execution, maintain visibility of dependencies and emerging issues, coordinate workstream interfaces, and consolidate the project's current delivery position.

Particular attention was given to the relationship between workstream progress and downstream readiness, ensuring that completed activities were not automatically treated as usable outputs until the relevant information, handoff or validation condition had been established.

2. WHAT WAS COMPLETED

The following project-management controls and deliverables were established or updated during Week 5:

- **Week 5 Project Task Tracker —** translated the project work structure into 21 actionable activities with ownership, dependencies, priorities, status, RAG condition, exceptions and next actions.
- **Project Execution & Progress Plan —** established the execution sequence, control approach, dependency management and progress-monitoring framework.
- **Updated Project Timeline —** translated the Week 4 roadmap into a more execution-oriented schedule position while maintaining dependency-based control.

- **Track Coordination Plan —** established the framework for managing workstream interfaces, information exchanges, handoffs, dependencies and cross-track decisions.
- **Summary of Major Project Issues —** consolidated the principal execution and coordination issues emerging during the transition into active delivery.
- **Cross-Track Collaboration Record —** documented direct interaction with the Data Analytics track and the project-management relevance of the information exchanged.
- **Week 5 Project Status Position —** consolidated available workstream evidence into an integrated view of progress, constraints, dependencies and transition requirements.

Collectively, these outputs moved the project from a primarily structural planning position toward a more evidence-informed execution baseline.

3. KEY FINDINGS AND DEVELOPMENT OUTCOMES

Week 5 produced the first substantive analytical evidence available for project-level consideration.

Data Analytics — Analytics & Insights

The Data Analytics track reported completion of data preparation and validation activities, exploratory analysis, development of six key performance indicators, and an executive Power BI dashboard.

The reported analysis identified an overall no-show rate, alongside observable patterns associated with previous no-show history, appointment timing, reminder channels and patient distance.

48.46% **Overall appointment no-show rate**	**77%** **No-show rate — patients 30–40 km away who received reminders**

One notable observation was a 77% no-show rate among patients in the 30–40 km distance group who received reminders. This was treated as an analytical signal requiring further investigation rather than as evidence that reminders themselves were ineffective, since the observed relationship may reflect differences in patient characteristics, behaviour or reminder selection.

The interaction with Data Analytics also identified booking lead time as an area requiring deeper analysis in the next phase.

For project evaluation, No-Show Rate and Show Rate were identified as practical baseline measures because they provide complementary indicators for assessing whether subsequent interventions produce measurable changes in appointment attendance.

From a Project Management perspective, the key outcome was therefore not simply the volume of analysis completed, but the establishment of an initial evidence base that can inform downstream predictive, engagement and intervention activities.

4. MAJOR CHALLENGES ENCOUNTERED

4.1 Downstream Dependencies Became More Visible

As workstreams moved into execution, it became clearer that several outputs would serve as inputs to other tracks. This increased the need to monitor not only whether an activity had started or finished, but whether its output was sufficiently ready for downstream use.

Management Response: Dependencies and handoff conditions were incorporated into the coordination framework, with particular attention to data outputs required by downstream analytical and intervention activities.

Current Position: ● **CONTROLLED**

4.2 Completion Did Not Always Equal Readiness

A completed activity does not necessarily represent a completed project input. Outputs may still require review, transfer, validation or acceptance before downstream work can rely on them.

Management Response: The project adopted a clearer distinction between activity completion, output production, handoff, acceptance and downstream enablement.

Current Position: ● **ACTIVE CONTROL**

4.3 Parallel Work Required Dependency-Based Coordination

The project could not be effectively managed as one strictly sequential chain. Some activities could progress independently, while others required defined upstream inputs.

Management Response: A controlled-parallel coordination model was maintained, allowing independent work to progress while preserving prerequisite and interface controls.

Current Position: ● **CONTROLLED**

4.4 Emerging Findings Required Careful Interpretation

Some analytical observations were sufficiently notable to influence project thinking but were not yet appropriate for use as causal conclusions.

The distance / reminder finding was the clearest example: the observed relationship requires further analysis before decisions are made regarding intervention design or reminder effectiveness.

Management Response: Emerging findings were treated as evidence requiring validation rather than conclusions requiring immediate implementation.

Current Position: ● **ACTIVE CONTROL**

4.5 Increasing Integration Requirements

Process & Operational Integration

As individual workstreams began producing tangible outputs, the need for those outputs to eventually operate as a coherent solution became more apparent.

Predictive intelligence, patient engagement, Generative AI support and operational workflow design will need to converge without creating conflicting requirements or unsupported assumptions.

Management Response: Process & Operational Integration was maintained as a key receiving interface, while cross-track dependencies and readiness conditions were incorporated into coordination control.

Current Position: ● **ACTIVE CONTROL**

5. KEY PROJECT MANAGEMENT DECISIONS

Decision 1 — Maintain Controlled-Parallel Execution

The project will not be managed as a single linear sequence. Workstreams may progress in parallel where prerequisites permit, while activities with defined dependencies remain controlled by their upstream conditions.

Rationale: *This prevents unnecessary waiting while protecting the integrity of dependent work.*

Decision 2 — Separate Completion From Downstream Readiness

Completion of an activity will not automatically be interpreted as readiness for downstream use.

Rationale: *This provides stronger control over quality, handoffs and dependency activation.*

Decision 3 — Treat Emerging Analytics as Evidence Requiring Validation

Observed relationships will inform further analysis and project discussion but will not be treated as causal findings without appropriate validation.

Rationale: *This reduces the risk of designing interventions around misleading or incomplete interpretations.*

Decision 4 — Establish No-Show Rate and Show Rate as Initial Evaluation Baselines

These measures will provide the initial reference point for assessing whether subsequent interventions produce measurable improvement in appointment attendance.

Rationale: *Both measures are directly aligned with the project's central outcome and provide a simple basis for before-and-after comparison.*

6. CHANGES FROM THE WEEK 4 APPROACH

Week 4 primarily answered:

> ***What are we solving, who is involved, how is the work structured, and how will it be governed?***

Week 5 moved toward:

> ***What is actually moving, what evidence is emerging, what depends on what, and what must be controlled before the next stage?***

The Principal Changes Were

Week 4 Position	Week 5 Execution Position
High-level work structure	**Actionable task-level execution**
Stakeholder and responsibility definition	**Active ownership and accountability**
Dependency identification	**Dependency and interface monitoring**
Initial risk identification	**Active issue and emerging-risk control**
Planned communication	**Cross-track coordination and handoffs**

Week 4 Position	Week 5 Execution Position
Project structure	**Evidence-informed delivery position**
Activity planning	**Progress, exception and readiness control**
Anticipated outputs	**Emerging evidence and downstream requirements**

The underlying project value chain remains:

Business Need → Patient Need → Evidence → Solution → Operational Adoption → Measurable Value

Week 5 strengthened the Evidence → Solution and Solution → Operational Adoption portions of this chain by introducing active execution and cross-track coordination.

7. CROSS-TRACK COLLABORATION

During Week 5, Project Management interacted directly with the Data Analytics — Analytics & Insights track.

The track shared its current data-preparation position, exploratory findings, KPIs, dashboard outputs and identified no-show patterns.

Project Management subsequently raised specific points for further consideration, including:

- The relationship between distance, reminder channels and previous no-show history;
- The potential significance of booking lead time;
- The relevance of cancellation patterns; and
- Which KPIs could provide an appropriate baseline for post-intervention comparison.

The interaction resulted in clearer visibility of the analytical position and identified areas for further validation in the next phase.

It also established No-Show Rate and Show Rate as practical baseline measures for subsequent project evaluation.

This interaction demonstrates the intended PM role as an integration layer between workstreams, rather than as a replacement for the specialist work performed within those tracks.

8. REMAINING WORK

The following areas remain important to the project's next delivery stage:

1. Complete and further validate the attendance / no-show analysis.
2. Progress predictive intelligence using appropriately validated analytical inputs.
3. Develop and refine patient-engagement intervention opportunities.
4. Define Generative AI support requirements, boundaries and escalation conditions.
5. Translate emerging outputs into operational workflow considerations.
6. Maintain visibility of cross-workstream dependencies and handoff conditions.
7. Validate analytical findings before they are used to support intervention decisions.
8. Continue updating project risks, issues, decisions and dependencies as evidence develops.

9. Prepare integrated outputs for subsequent testing and validation.
10. Maintain schedule and milestone control as downstream activities become execution-ready.

9. WEEK 6 PRIORITIES

Week 6 should move the project further from individual workstream execution toward validated outputs and increasing integration readiness.

The principal Project Management priorities are to:

- Consolidate and monitor the maturity of workstream outputs;
- Track further validation of attendance and no-show findings;
- Monitor dependencies affecting predictive intelligence and patient engagement;
- Maintain Generative AI progress within approved information and escalation boundaries;
- Strengthen output handoffs and downstream-readiness controls;
- Monitor emerging integration requirements;
- Update risks, issues, decisions and dependencies based on new evidence;
- Maintain alignment between the project timeline and verified execution progress;
- Assess readiness for testing and integrated solution refinement; and
- Establish a clearer basis for measuring project outcomes against the agreed attendance baseline.

10. MANAGEMENT ASSESSMENT

Week 5 marked a substantive transition from project establishment to controlled execution.

The project now has greater visibility of task-level activity, workstream ownership, dependencies, analytical evidence, emerging issues and cross-track interfaces. More importantly, progress is increasingly being assessed not only by whether work has been performed, but by whether the resulting output is reliable, transferable and ready to enable downstream activity.

The principal management challenge going forward is therefore no longer simply initiating work. It is maintaining alignment as multiple specialist tracks produce outputs that must eventually operate as one coherent solution.

At the close of Week 5, the project is assessed as:

● AMBER — CONTROLLED EXECUTION

No unresolved project-level blocker or formal completion-date variance has been established from the current evidence.

The immediate management priority for Week 6 is to strengthen validation, control dependencies and convert emerging workstream outputs into integration-ready project inputs.

Week 5 Closing Position

> ***Structure has been established. Execution is underway. Evidence is emerging. The next control challenge is integration.***

End of Week 5 Project Summary